

1. Administrative & Study Identification

Full Study Title: A Study of Participants in China With Non-Small-Cell Lung Cancer That is Unable to be Treated With Surgery
Short Title/Acronym: NSCLC Observational Cohort Study China
Protocol Number: ML44633 **NCT Number:** NCT05872763 **Sponsor Name:** Hoffmann-La Roche **Investigational Product:** N/A (Observational Study - Real-world first-line anti-cancer treatments) **Phase of Development:** N/A (Observational) **Medical Monitor:** Hoffmann-La Roche Clinical Operations Lead **Overall Status:** Recruiting

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To describe the clinical practice and evaluate the long-term survival benefits (Median Overall Survival) of patients newly diagnosed with unresectable stage IIIB/IIIC/IV Non-Small-Cell Lung Cancer (NSCLC). **Secondary Objectives:** To explore the utilization of biomarker tests (EGFR, ALK, ROS1, BRAF, NTRK, KRAS, MET, RET, ERBB2, PD-L1), assess potential economic impacts on patients in the real world, and summarize safety-related events.

2.2 Background & Rationale To gather comprehensive real-world evidence regarding treatment patterns, biomarker testing rates, and long-term clinical outcomes in patients with unresectable stage IIIB/IIIC/IV NSCLC in China. Because these patients are unable to be treated with surgery, this study is essential to understand standard real-world management and its impact on overall survival.

3. Study Design & Methodology

3.1 Design Framework Study Type: Observational (Multicenter cohort study with primary prospective and retrospective data collection) **Time Perspective:** Other (Prospective and Retrospective) **Control Type:** N/A **Blinding:** Open-label **Randomization:** N/A **Sampling Method:** Non-Probability Sample

3.2 Planned Enrollment & Duration Target N (Estimated Enrollment): Variable (Targeting a relatively even number of patients across selected sites) **Number of Sites:** 7-10 sites across 7 geographic regions in China (Northwest, Northeast, North, Southwest, South, Central, East) **Estimated Study Start:** 11-Aug-2023 **Estimated Primary Completion:** 21-Sep-2026 **Estimated Study Completion:** 21-Sep-2026

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria Cohort 1 (Retrospective cohort): 1. Participants who have diagnosed with unresectable stage IIIB/IIIC/IV NSCLC. 2. Participants who have been diagnosed six months before the start of the study are required to have two complete tumor assessments within at least 6-month intervals after the diagnosis. Participants who were newly diagnosed within six months before the start of the study are required to have complete baseline information. 3. Participants who have received standard first-line treatment defined by the Chinese Society of Clinical Oncology (CSCO) and National Comprehensive Cancer Network (NCCN) guidelines.

Cohort 2 (Prospective cohort): 1. Participants who are newly diagnosed with unresectable Stage IIIB/IIIC/IV NSCLC after the start of the study. 2. Participants who are able to be followed up by the participating site. 3. Participants planned to receive first-line anti-cancer treatment targeting unresectable stage IIIB/IIIC/IV NSCLC in the study site after study initiation.

4.2 Exclusion Criteria 1. Participants who have received prior systematic treatment for unresectable Stage IIIB/IIIC/IV NSCLC. 2. Participants who have participated in any anti-cancer, regimen-specified clinical study of first-line treatment for unresectable Stage IIIB/IIIC/IV NSCLC.

4.3 Demographic Criteria * **Ages Eligible for Study:** All ages (Child, Adult, Older Adult) * **Sexes Eligible for Study:** All * **Accepts Healthy Volunteers:** No

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Real-world standard of care dosages. First-line treatment will be reported by class of treatment. **Route of Administration:** Oral, IV, etc., depending on the standard of care therapy prescribed. **Duration of Treatment:** Dependent on clinical response and physician discretion. Follow-up continues until death, lost-to-follow-up, or 12 months after the last participant is enrolled in cohort 2.

5.2 Concomitant & Prohibited Therapy Permitted: Routine concomitant therapies per real-world clinical practice guidelines. **Prohibited:** Participation in regimen-specified interventional clinical studies.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures	:---
:---	:---	Screening/Baseline	Day 0 Retrospective medical record review, baseline tumor assessments, biomarker

tests evaluation (EGFR, ALK, ROS1, etc.) | | **Interim Follow-up** | Real-world intervals | Collection of prospective treatment data, response assessments, safety-related events, economic impact tracking | | **End of Study** | Follow-up End | Final vital status (Survival analysis), cause of death, time on first-line treatment evaluation |

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Median Overall Survival (OS) - Defined as the time from the date of the first administration of anti-cancer treatment until the date of death from any cause.

Measurement Method: Real-world survival tracking via medical records and routine follow-up.

7.2 Secondary & Exploratory Endpoints * Utilization rates and condition of biomarker tests (EGFR, ALK, ROS1, BRAF, NTRK, KRAS, MET, RET, ERBB2, PD-L1). * Duration (in weeks) on the first line of treatment (summarized using median, minimum, and maximum values). * Number of Cycles of Anti-cancer Treatment. * Safety-related events profile (Number of participants with at least one adverse event, with severity determined according to National Cancer Institute Common Terminology Criteria for Adverse Events, Version 5.0 (NCI-CTCAE v5.0)). * Potential real-world economic impact on patients.

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Standard observational reporting of safety-related events based on medical record reviews. **Serious Adverse Events (SAEs):** Reported according to local observational registry requirements. **Data Monitoring Committee (DMC):** Utilizes standard observational data oversight to ensure scientific integrity.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Observational cohort sets (prospective and retrospective cohorts). **Sample Size Justification:** A non-probability sample targeting 7-10 regional sites to ensure geographic representation across China rather than strict hypothesis testing. **Handling of Missing Data:** Standard handling for observational real-world evidence, minimizing bias in survival evaluations.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP), the Declaration of Helsinki, and Chinese observational study regulations. **Monitoring Plan:** Periodic data extraction from medical records and electronic health systems across the 7-10 involved sites. **Audit Trail:** Electronic Case Report Form (eCRF) capturing prospective treatment updates, tumor assessments, and biomarker inputs.

11. Study Identifiers & Governance

Primary ID: NCT05872763 **Protocol Number/Other ID:** ML44633
Registry Number: NCT05872763 **Sponsor/Lead Organization:** Hoffmann-La Roche **Study Title:** A Study of Participants in China With Non-Small-Cell Lung Cancer That is Unable to be Treated With Surgery
Official Regulatory Title: A Study of Participants in China With Non-Small-Cell Lung Cancer That is Unable to be Treated With Surgery **Study Condition:** Carcinoma, Non-Small-Cell Lung

12. Clinical Framework (Oncology Specific)

Primary Condition: Non-Small-Cell Lung Cancer (NSCLC) / Carcinoma, Non-Small-Cell Lung **Diagnosis Threshold:** Unresectable stage IIIB/IIIC/IV NSCLC **Medical Methodology:** Standard first-line treatment (e.g., PD-1/PD-L1 inhibitors, chemotherapy) guided by CSCO and NCCN guidelines **Optimization Target:** Improvement of Median Overall Survival (OS) and biomarker-driven therapy alignment

13. Study Procedures & Methodology

Management Pathway: Real-world standard clinical pathway for advanced NSCLC **Education Protocol (HCP):** Site initiation and observational data collection training **Education Protocol (Patient):** N/A (Observational standard-of-care follow-up) **Quality Audit Mechanism:** Data verification against primary medical records and pathology reports for tumor/biomarker profiles

14. Observation & Follow-Up Schedule

Total Duration: Until 12 months after the last participant is enrolled in cohort 2, or until death/loss to follow-up **Onsite Visit Cadence:** Aligned with real-world clinical follow-up intervals for tumor assessments **Remote Monitoring Frequency:** N/A (Data abstraction from records) **Checklist Review Interval:** Continuous abstraction tracking through the estimated completion date

15. Sign-off & Approval

Role	Name	Signature	Date	:---	:---	:---	:---	
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]					
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]			Lead		
Statistician	[Name]	_____	[DD-MMM-YYYY]					